



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : ناهد محمد فوزى طه
المحافظة : القاهرة
الحى / المدينة : حلوان
العنوان : 0
رقم البطاقة : 26412200102601 رقم التليفون : 01148483392
العمر (تاريخ الميلاد) : 20-12-1964 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 21-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
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التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.
تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: ناهد محمد فوزي طه National ID: 26412200102601	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bone scintigraphy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



ناهد
محمد فوزي طه حسن
١٨ ش مفتاح - عرب غنيم
حلوان - القاهرة

١٩٦٤/١٢/٢٠ ٢٦٤١٢٢٠٠١٠٢٦٠١

J10250475

٢٠٢٢/٠٣ ٢٦٤١٢٢٠٠١٠٢٦٠١

ربة منزل
أنثى مملعة أرملة

٢٠٢٩/٠٣/١٦ البطاقة سارية حتى






مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

سنة المطرية
١١٠٠
١١٠٠
١١٠٠
١١٠٠

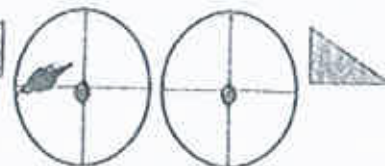
0.01-10-14

Postoperative
MDT FORM

$$Rt$$

BCS + LCN

<u>Name</u>	Wine & Oil
<u>Age</u>	5.6 Post
<u>MRN</u>	C7E1CC-15
<u>ID</u>	1151111111
<u>Co</u>	Dm
<u>morbidity</u>	C. 1111111111



Clinical Data

Bx:	IJC II	SM:
IHC:	ER: 8/8 PR: 5/8 HER2: <u>1+</u> Ki67: <u>15%</u>	Mets Workup: CT Can - free
T₁ N₂ M P 1/16. 4 GCC		
COMMENT: Bone scan Echo - Labs LGR 63%		

MDT OUTCOME & PLAN

For Adjurat. AC X 4 yds $\rightarrow$ Tarsal weekly
X 12 wk
Followed by PORT
- A-I 15y + prola 16 mi

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
<i>[Signature]</i>	<i>[Signature]</i>	<i>[Signature]</i>	

9/11/18

09/14

109912



Patient Name : الأستاذ / ناهد محمد فوزى طه حسن	Request No. : 137702
Patient No. : 26412200102601	Request Date : 20/11/2021
Path No. : 2850/21	Verification Date : 30/11/2021
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 56 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data:

Right breast cancer.

Nature of the specimen:

Right WLE and axillary lymph nodes specimens.

Gross Examination:

- Procedure: Right wide local excision specimen; measured 7x5x2 cm, labeled as LL, SS, DD
- Tumor extent
 - o Number: One well defined rubbery nodular grayish white mass, another ill defined firm grayish white mass, away from each other by 0.7 cm and separated with breast tissue
 - o Size: Well defined mass measured 1.5x1 cm, ill defined firm mass measured 1.7x1 cm.
- Distance from margins: Least margin was 1.5 cm (superior margin)
- Lymph nodes number and size: Axillary fat dissection revealed 16 nodules up to 2 cm.
- Frozen section: Results: Free surgical margins.

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): II
- Angioinvasion: Positive
- Ductal carcinoma in situ: Positive.
- Lymph node involvement: Positive metastatic deposits in 9/16 axillary nodes with extracapsular extension.
- Resection Margins: Free.
- Histologic response to therapy: N/A
- Additional pathologic findings: Associated fibroadenoma.

Diagnosis:

Verified by :

walaa ghoniem



Patient Name : الأستاذ / ناهد محمد فوزى طه حسن :	Request No. : 137702
Patient No. : 26412200102601	Request Date : 20/11/2021
Path No. : 2850/21	Verification Date : 30/11/2021
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 56 Y	Department : Out Patient
Referreing Doc :	Registered by : .marwa mohamed youss

Pathology Report

Right breast mass; Right wide local excision and axillary nodes excision:

- Invasive duct carcinoma, grade II, with associated DCIS component.
- Negative for perineural invasion
- Positive for lymphovascular embolization.
- Free all surgical margins
- Positive metastatic deposits in 9/16 axillary lymph nodes with extra capsular extension.
- T1 N2 Mx.

Prof. Dr. Aml Hareedy
Dr. Walaa Ghoniem.

Verified by :
walaa ghoniem



Patient Name	: الأستاذة / ناهد محمد فوزى طه حسن	Request No.	: 138522
Patient No.	: 26412200102601	Request Date	: 05/10/2021
Path No.	: 2320/21	Verification Date	: 23/10/2021
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 56 Y	Department	: Out Patient
Referring Doctor:		IHC No.	:

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass

Nature of the specimen: Biopsy

Gross Examination: Five grayish white and fatty tissue cores, 1.5 cm each. Totally submitted.

- Procedure: Tru cut
- Localization: Right
- Additional pathologic findings: N/A
- Immunohistochemistry technique: Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (SP1), PR clone (1E2), HER-2 CLONE (4B5), & Ki67 (30-9)

Microscopic examination:

- Histological type: Invasive duct carcinoma.
- Grade : G II
- Angioinvasion: No
- Ductal carcinoma in situ: No

Diagnosis:

Right breast mass, Core biopsy;
• Invasive duct carcinoma, grade II

• Biomarkers:

- o ER: Strong 90% nuclear reaction (Allred score 8/8)
- o PR: Moderate 30% nuclear reaction (Allred score 5/8)
- o Her2: No membrane reaction (score 0)
- o Ki67: 15% nuclear reaction

Prof. Dr. Amal Hageedy
Dr. Mona Mohamed



Patient Name: Nahid Mohamed Fawzy

Date: 29/1/2022

Tc-99m MDP bone scan.

Desired information : skeletal surveying to detect multiple lesions.

Technique:

Delayed Anterior and posterior whole body scan 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP.

Report:

Delayed Static Phase:-

- Degenerative changes are noticed over dorso-lumbar vertebrae.
- No definite hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
- Both kidneys are normally seen

Conclusion:

- *The surveyed skeleton is free of osseous deposits.*

Thanks for reference

Nuclear medicine Unit

Dr.Omnia Talaat , MD

Dr.Sherine Mahdy,MD



Name: Nahed Mohammed Date: 31-10-2021

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Right breast UOQ two adjacent interconnected irregular speculated mass lesions the largest measuring 1.6 cm in diameter
- Right axillary level I and II enlarged lymph nodes the largest measures 2.3 X 1.2 cm.
- No sizable pulmonary nodules.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels

2. Abdomen and pelvis:

- The liver is enlarged with homogenous low CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The scanned skeleton shows L5 lytic lesion with vertical striations mostly hemangioma.



OPINION:

- ***Right breast malignant looking mass lesions with pathological ipsilateral axillary lymph nodes as described.***
- ***No definite pulmonary, hepatic or osseous metastatic deposits.***

Dr. Mohammed Mohsen, MSc.

Dr. M. Gomaa, M.D.



Patient Name: Nahed Mohamed Fawzy Taha Hassan	Date : 02/07/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 59 Year(s)
Medical ID: 4038/2021	Pat. ID: 32395

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND;

Indication:

Right breast conserving therapy

Report:

- Entirely fatty breast parenchyma (ACR A)

Right breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Left breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post right BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Best regards,
Dr. Eman Abdel Razik, MSc, FEBR
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
02/07/2024 1:19 PM



Patient Name: Nahed Mohamed Fawzy Taha Hassan	Date : 28/12/2023
Exam : MAMMOGRAPHY BOTH	Age : 59 Year(s)
Medical ID: 4038/2021	

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had rt CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Right breast focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Dr

Yours truly ,

Dr/Asmaa Zeinhom, Msc

Not Approved

مركز الامراض الصدرية
مركز سرطان الثدي

Patient name: ناهد محمد فوزي

Date: 28/09/2021

Combined bilateral breast mammography and complementary ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- The breasts are almost entirely fatty (ACR A)
- Right breast shows spiculated hyperdense mass in UOQ measuring about 20 x15 mm, seen by complementary ultrasound at 10-11 o'clock irregular shape spiculated hypoechoic mass with internal vascularity measuring about 21x14
- Right UOQ well circumscribed oval shaped dense mass seen by US as well defined hypoechoic mass measured about 18x25mm located 1cm away from the suspicious mass
- Left breast shows no suspicious masses or cysts.
- Normal right skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Right multiple enlarged axillary LNS with nonuniform cortical thickening reacting about 8mm and effaced hilum
- Left breast 3O'clock dermal thickening and collection , measured about 3x1.5cm

Opinion:

- Right breast UOQ suspicious mass (BIRADS 4c) for histopathological correlation
- Left breast dermal inflammatory process (BIRADS2)

Yours truly

Dr\ Walaa Abdel _aziz

Breast imaging and intervention
consultant